

MOVEMENT IS MEDICINE.

YOUR TYPE 2 DIABETES GUIDE

How to use exercise to improve blood glucose control, energy and long-term health.



Created by Accredited Exercise Physiologists

SOUTH HOBART · LINDISFARNE

1.7M

Australians living with diabetes

85-90%

of diabetes cases are type 2

210

minutes a week of moderate exercise recommended

ONE OF YOUR BEST TOOLS.

Exercise works on blood glucose directly — and it starts working the same day you do it.

280

Around 280 Australians develop diabetes every day, roughly one every five minutes. Type 2 is the large majority of cases.

1

MUSCLES USE UP SUGAR

Working muscles pull glucose out of your blood to use as fuel.

2

INSULIN WORKS BETTER

Regular activity makes your body more sensitive to the insulin you have.

3

YOUR HEART BENEFITS TOO

Lower blood pressure and cholesterol, where diabetes raises the risk most.

MYTH VS FACT

MYTH

Diabetes means I should avoid strenuous activity.

FACT

With the right guidance, most people with type 2 diabetes can safely do both walking and strength exercise.

MYTH

Medication and diet are enough on their own.

FACT

Exercise is the third main tool, and it acts on blood glucose directly.

MYTH

Any activity is the same for diabetes.

FACT

Mixing walking with strength training beats doing either one alone.

TALK TO YOUR AEP OR DOCTOR FIRST IF

- ✓ You use insulin or medication that can cause a hypo
- ✓ You have nerve, eye or kidney complications
- ✓ You have heart disease or are new to exercise

The target: at least 210 minutes of moderate exercise a week (or 125 harder minutes), plus strength training — and never more than two days in a row off.

FOUR THINGS IT DOES FOR YOU.

Regular movement changes how your body handles glucose — immediately after a session, and over the long term.

1

LOWERS BLOOD GLUCOSE

Your muscles use blood sugar for fuel, bringing levels down during and after activity.

2

BUILDS WORKING MUSCLE

Strength training gives you more muscle to store and burn glucose, which helps long-term control.

3

PROTECTS YOUR HEART

Lower blood pressure and cholesterol and better fitness, where diabetes raises risk most.

4

LIFTS ENERGY AND MOOD

Regular activity is linked to better energy, better sleep and lower risk of depression.

STAYING SAFE AROUND EXERCISE

Check your glucose first if you use insulin

Carry a fast-acting carb in case of a low

Stay well hydrated, especially in warm weather

Check your feet if sensation is reduced

ESSA recommends mixing walking-style exercise with strength training, spread through the week with no more than two rest days in a row.

WEEKS 1-2

Building a consistent routine you can sustain

WEEKS 3-8

More energy and often better glucose readings

ONGOING

Long-term heart and metabolic health benefits

START HERE.

A mix of aerobic and resistance exercises to build a well-rounded routine. Your Exercise Physiologist will individualise intensity and progression to you.

1

BRISK WALKING

Walk at a pace that raises your breathing rate while still able to talk, building duration over time.

10–30 minutes, most days

2

SIT TO STAND (SQUATS)

From a sturdy chair, stand up fully then lower back down with control, building lower body strength.

2–3 sets of 8–12 reps

3

WALL PUSH-UPS

Hands on a wall at shoulder height, bend and straighten your elbows to build upper body strength.

2–3 sets of 10–12 reps

4

RESISTANCE BAND ROW

Standing or seated, pull a resistance band towards you, squeezing your shoulder blades together.

2–3 sets of 10–12 reps

5

STANDING MARCHING

March on the spot, lifting knees to a comfortable height, to build aerobic fitness indoors.

2–3 sets of 30–60 seconds

6

STEP-UPS (LOW STEP)

Step up and down onto a low, sturdy step, alternating leading leg for a combined effect.

2 sets of 8–10 reps each leg

7

SEATED LEG EXTENSIONS

Seated in a sturdy chair, straighten one knee at a time to work the front of your thigh.

2 sets of 10–12 reps each leg

8

GENTLE FULL-BODY STRETCHING

A short stretching routine for major muscle groups, to support mobility and recovery.

5–10 minutes, after activity



BEFORE YOU START

If you use insulin or medications that can cause low blood glucose, check your levels before and after exercise as advised by your care team, and carry a fast-acting carbohydrate source. Stop if you feel shaky, dizzy or unwell, and check with your doctor or Accredited Exercise Physiologist before starting, especially if you have diabetes-related complications.

210 MINUTES A WEEK.

Aiming for at least 210 minutes of moderate activity per week, combined with resistance training, spread across the week.

DAY	FOCUS	WHAT TO DO
MON / WED / FRI	Resistance	Sit to Stand + Wall Push-Ups + Resistance Band Row + Step-Ups
TUE / THU / SAT	Aerobic	20–30 minutes of brisk walking or standing marching
EVERY DAY	Recovery	Gentle full-body stretching, especially after activity
EVERY DAY	Consistency	No more than two consecutive days without activity

WEEK 1-2

Focus on building a consistent, comfortable routine and correct technique.

WEEK 3-8

Gradually increase duration and resistance as strength and confidence build.

BEYOND

A supervised program helps fine-tune intensity to your blood glucose response and goals.

FROM YOUR HEP TEAM

A PROGRAM BUILT AROUND YOUR NUMBERS

Our Accredited Exercise Physiologists work with your blood glucose targets, medications and goals to build a safe, effective exercise plan, in partnership with your GP or endocrinologist.

BOOK ONLINE

www.hobartexercisephysiology.com.au

WHERE THIS COMES FROM.

Every recommendation in this guide is drawn from peer-reviewed research and professional body position statements.

01 Hordern, M.D., Dunstan, D.W., Prins, J.B., Baker, M.K., Fiatarone Singh, M.A. & Coombes, J.S. (2012) 'Exercise prescription for patients with type 2 diabetes and pre-diabetes: a position statement from Exercise and Sport Science Australia', *Journal of Science and Medicine in Sport*, 15(1), pp. 25–31.

02 Colberg, S.R., Sigal, R.J., Fernhall, B., Regensteiner, J.G., Blissmer, B.J., Rubin, R.R., et al. (2010) 'Exercise and type 2 diabetes: joint position statement', *Diabetes Care*, 33(12), pp. e147–e167.

03 Colberg, S.R., Sigal, R.J., Yardley, J.E., Riddell, M.C., Dunstan, D.W., Dempsey, P.C., et al. (2016) 'Physical activity/exercise and diabetes: a position statement of the American Diabetes Association', *Diabetes Care*, 39(11), pp. 2065–2079.

04 Exercise Right (2024) *Exercise Right for Type 2 Diabetes*. Exercise & Sports Science Australia. Available at: essa.org.au (Accessed: 2026).

A NOTE ON ADVICE

This guide is for general educational purposes and is not a substitute for individual medical advice. Speak with your doctor, endocrinologist or an Accredited Exercise Physiologist before starting a new exercise program, particularly if you use insulin or other glucose-lowering medications.